

CVS Exit and Reallocation: Approve the Exit, Repair the Portfolio

An unbiased assessment of closing the remaining CVS Health position and reallocating to VXUS, AVUV, or higher-conviction individual names.

Prepared July 17, 2026 Long-only retirement portfolio Approximately 10-year horizon

Market data through July 17, 2026

Recommendation: exit the remaining CVS position

PM JUDGMENT The decision is driven more by **portfolio opportunity cost** than by a bearish view of CVS. The turnaround is progressing, but at roughly \$106.50 the shares already discount meaningful execution. The portfolio has clearer structural needs: international diversification and small-cap value exposure.

Approve

Reallocate 75% to VXUS and 25% to AVUV. Do not redirect these proceeds to another concentrated U.S. stock.

REMAINING CVS

0.92%

\$14,461 market value
in latest portfolio file

CVS VALUATION

14.4×

Reference price /
midpoint of 2026
adjusted EPS guide

DIRECT VXUS

0.27%

Far below the long-run
international objective

DIRECT AVUV

0.42%

Useful size/value
diversifier, but not
sector-neutral

1. Decision hinge

CVS is no longer obviously mispriced enough to justify keeping a sub-1% turnaround position when the same capital can improve portfolio construction.

The strongest case for holding CVS

- Q1 2026 adjusted EPS rose to \$2.57 and management raised full-year adjusted EPS guidance to \$7.30-\$7.50.
- Health Care Benefits margin recovery is real: MBR improved to 84.6% from 87.3%, and segment adjusted operating income rose 52.6%.
- Management targets mid-teens adjusted EPS CAGR through 2028; success could support attractive returns even without a heroic multiple.
- The \$2.66 annualized dividend provides a roughly 2.5% yield at the reference price.

The stronger case for exiting

- Some Q1 improvement came from the absence of a prior-year \$448 million premium deficiency reserve and favorable reserve development - not solely sustainable operating improvement.
- Health Services adjusted operating income fell 7.1%, while Pharmacy & Consumer Wellness fell 8.8% amid client price concessions and reimbursement pressure.
- CVS carried about \$63.1 billion of current and long-term debt at March 31; Fitch and S&P had negative rating outlooks.
- At 14.4× guidance, CVS is a fair-priced execution bet rather than the deep-value setup that originally justified the turnaround position.

Bottom line: CVS can still work. The portfolio does not need it to work. A 0.92% position is too small to drive results, yet the thesis demands active monitoring across medical utilization, Medicare Advantage pricing, PBM economics, retail reimbursement, debt and regulation.

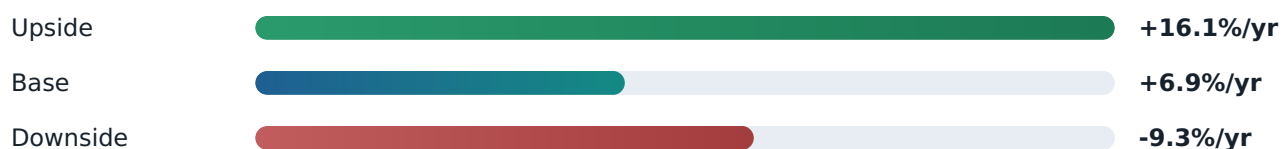
2. CVS stock thesis at today's price

EVIDENCE	WHAT IT SAYS	PM INTERPRETATION
2026 adjusted EPS guide	\$7.30-\$7.50 after a \$0.30 increase	Recovery is ahead of the original plan.
Q1 Health Care Benefits	MBR 84.6%; adjusted operating income +52.6%	Core insurance repair is advancing.
Health Services / Retail	Adjusted operating income -7.1% / -8.8%	Improvement is not broad-based.
Balance sheet	\$2.58B current debt + \$60.53B long-term debt	Limits capital-allocation flexibility.
Capital returns	No shares repurchased in Q1; \$0.665 quarterly dividend	Debt and operations remain ahead of buybacks.
Reference valuation	\$106.50 / \$7.40 = 14.4× adjusted EPS	Reasonable, but not a large margin of safety.

Illustrative return cases through year-end 2028

ANALYST MODEL These are scenario assumptions, not consensus estimates. Returns include an estimated \$6.70 of cumulative dividends over approximately 2.46 years.

CASE	2028 EPS	EXIT P/E	PRICE TARGET	ANNUALIZED TOTAL RETURN	WHAT MUST HAPPEN
Upside	\$9.80	15.0×	\$147	+16.1%	Management's mid-teens growth target is delivered and quality concerns recede.
Base	\$8.80	13.5×	\$119	+6.9%	Benefits recovery continues, offset by PBM/retail pressure and leverage.
Downside	\$7.00	11.0×	\$77	-9.3%	Medical costs reaccelerate, guidance slips, and the multiple compresses.



The skew is acceptable but not compelling enough to override the portfolio-fit argument. CVS would become more interesting again near **12× current guidance, or roughly \$89**, assuming the fundamental thesis had not weakened.

3. Reallocation alternatives

ALTERNATIVE	WHAT IT ADDS	PRIMARY RISK	PORTFOLIO FIT	VERDICT
VXUS	Broad developed and emerging ex-U.S. exposure; 0.05% expense ratio	Currency, geopolitical and structurally lower-profitability exposures	Direct position is only 0.27%; closes the largest allocation gap	First priority
AVUV	Small-cap value and profitability tilts across 792 holdings	Higher volatility; 28.6% financials and 14.3% energy; strong recent run	Useful style/size diversification, but overlaps existing financial exposure	Second priority
MSFT / META / AVGO	Higher-quality economics and clearer secular growth than CVS	Adds to an already large technology/semiconductor concentration	Raises single-stock and factor concentration rather than repairing it	Do not use these proceeds
PEP / AMT	Defensive or income-oriented quality exposure	Slower growth / rate sensitivity; still single-company risk	More balanced than adding technology, but less valuable than filling allocation gaps	Use future contributions

Important nuance on AVUV: it diversifies by market capitalization and valuation factor, not fully by sector. Its 28.6% financials weight matters because direct financials and payments already represent roughly 26% of the portfolio. That is why AVUV receives 25%, not 50%, of the CVS proceeds.

4. Recommended implementation

75% VXUS · ~\$10,846

25% AVUV · ~\$3,615

HOLDING	BEFORE	INCREMENT	PRO FORMA	ROLE
CVS	0.92%	-0.92%	0.00%	Remove single-company turnaround risk
VXUS	0.27%	+0.68%	0.95%	International core diversifier
AVUV	0.42%	+0.23%	0.65%	Small-value satellite

Execution: because the transaction is inside the 401(k), there is no capital-gains tax or tax-loss-harvesting benefit to waiting. If behavioral comfort favors staging, deploy half of each ETF allocation immediately and the balance over the next 30-60 days; otherwise, immediate rebalancing is defensible. Avoid making the strategic decision contingent on CVS's August 5 Q2 earnings event.

Why not 100% VXUS? That is a defensible alternative - and the cleanest answer if international diversification is the only objective. The 75/25 split preserves progress on the planned AVUV build while recognizing that VXUS is the larger portfolio need.

5. Action rules and disconfirmers

TRIGGER	OBSERVABLE THRESHOLD	ACTION
CVS re-entry watchlist	Price at or below ~12× current guide (~\$89), with guidance intact	Re-underwrite as a starter; do not buy solely because it is cheaper
CVS thesis improves	Q2/Q3 guide raised; HCB MBR remains near 85%; Health Services and retail profit stabilize	Update opportunity-cost comparison before any re-entry
CVS thesis breaks	Adjusted EPS guide below \$7.30, MBR reaccelerates, or debt rating loses investment grade	Keep the exit; remove from active buy list pending full re-underwrite
VXUS build	Direct international remains well below long-run target	Prioritize future contributions until total international exposure is mapped and on plan
AVUV build	Financial exposure remains elevated or AVUV exceeds its small-value target	Slow additions; use VXUS or broad international exposure instead
Portfolio concentration	New money would increase already-large technology or financial active bets	Do not redirect CVS proceeds to AVGO, META, MSFT, GS or JPM

6. Final PM assessment

Approve the full CVS exit. This is a portfolio-quality upgrade even if CVS subsequently rises. The company's recovery remains credible, but the stock no longer offers enough valuation asymmetry to compensate for its leverage, regulation, reimbursement pressure and monitoring burden. VXUS and AVUV are not guaranteed to outperform CVS; they are superior uses of this particular capital because they improve the portfolio's diversification and reduce reliance on a single turnaround.

The correct benchmark for this decision is not CVS's historical cost basis. The latest file shows a roughly 42% unrealized loss, but the 401(k) provides no tax benefit from realizing it. The only relevant question is which asset offers the best prospective risk-adjusted contribution from today forward.

Decision classification: Exit CVS; fund VXUS first and AVUV second; retain no CVS stub. Confidence is **medium-high**. The conclusion is robust to a reasonably positive CVS outlook because the binding constraint is portfolio fit, not a prediction that CVS must decline.

Sources and methodology

- User portfolio workbook received July 14, 2026: portfolio value approximately \$1.59 million; CVS \$14,461 (0.92%); VXUS 0.27%; AVUV 0.42%; CVS cost basis \$25,106. Portfolio figures may differ slightly from live values.
- CVS Health Q1 2026 earnings release: revenue, EPS, guidance and cash-flow guidance.
- CVS Health Q1 2026 Form 10-Q: segment results, MBR, debt, ratings, dividends and repurchases.
- CVS Health 2025 Investor Day release: mid-teens adjusted EPS CAGR target through 2028.
- CVS Health stock information: \$106.50 closing reference price on July 16, 2026.
- Avantis AVUV fund page and June 30, 2026 fact sheet: expense ratio, holdings, sector weights and performance.
- Vanguard VXUS fund page: mandate, expense ratio and current performance data.
- Scenario values are analyst assumptions. Annualized total returns use a \$106.50 reference price, approximately 2.46 years to December 31, 2028, and \$6.70 estimated cumulative dividends. Adjusted EPS excludes items defined by management and should not be treated as GAAP EPS.

Prepared as portfolio-manager analytical support, not a guarantee of results. Market prices and portfolio weights change. Reconfirm live prices, available plan investment options and current portfolio weights before execution.